MIDWIFERY QUESTIONS

PART II: SHORT ANSWER QUESTIONS. **MARKS**.

Q:1: a)State three (3) causes of brow presentation. 3

b)List four(4) complications of brow presentation. 2

Q: 2: State five (5) vital signs that are considered in APGAR scoring. 5

Q: 3: Briefly explain the four (4) stages of an eclamptic fit . 8

Q: 4: List six (6) complications of polyhydramnios. 3

Q: 5: State four (4) minor disorders of pregnancy. 4

Q: 6: State five (5) aims of an obstetric examination. 5

Q: 7: Outline five characteristics of a true female pelvis. 5

Q: 9: Draw a diagram showing the female external genital organs. 5

PART III: LONG ANSWER QUESTIONS (ESSAY) **MARKS**

Q: 1: Baby Jewel is admitted to the special baby care unit with a diagnosis of prematurity.

a) Define prematurity. 1

b) State five (5) causes of prematurity. 5

c) Describe the management of baby Jewel from admission until discharge. 12

d) State two (2) preventive measures of prematurity. 2

Q: 2: Mrs. X, gravida 3 is diagnosed with shoulder dystocia in the second stage of labour.

a) Define shoulder dystocia. 1

b) State five (5) risk factors of shoulder dystocia. 5

c) Using Mc Robert's maneuver, describe the management of Mrs. X up to 24 hours following delivery. 11

d) List four (4) complications that may be associated with shoulder dystocia 2

PART II: SHORT ANSWER QUESTIONS. **MARKS**

Q: 1: State two (2) physiological changes that occur in each of the following systems during pregnancy.

a) Cardiovascular system. 2

b) Skeletal system. 2

Q: 2: Draw a well labeled diagram of the female pelvis illustrating the landmarks of the pelvic brim. 5

Q: 3: State five (5) minor disorders of pregnancy related to the digestive system. 5

Q: 4: State four (4) premonitory signs of labour. 4

Q: 5: State four (4) observations made by the midwife during the second stage of labour that indicate good progress . 4

Q: 6: State five (5) vital signs that are considered in APGAR scoring. 5

Q: 7: State three (3) immunological adaptations of the newborn. 3

Q: 8: Outline five (5) factors responsible for infant morbidity and mortality. 5

Q: 9: State five (5) major complications of puerperium. 5

PART III: LONG ANSWER QUESTIONS (ESSAY) **MARKS**

Q: 1: Master Xuma is born at 34 weeks gestation. On the third day of life he develops jaundice.

a) State two (2) types of jaundice. 2
b) State four (4) causes of Master Xuma's condition. 4
c) Describe the management of Master Xuma till discharge. 13
d) List two (2) complications Master Xuma is likely to develop. 1

Q: 2: Mrs. Bela aged 38 years, para 2+0, gravida 3 is admitted in the maternity ward with a diagnosis of breech presentation in labour.

a) Define breech presentation. 1

b) State four (4) types of breech. 4

c) Describe the management if Mrs. Bela till discharge. 12

d) State three (3) complications of breech presentation in labour. 3

SHORT ANSWER QUESTIONS.

		MARKS
Q: 1:	State three (3) objectives of focused antenatal care.	3
Q: 2:	Draw a well labeled diagram of the fetal skull showing landmarks of obstetric importance.	5
Q: 3:	State five (5) predisposing factors to cord prolapse.	5
Q: 4:	Explain the components of active management of third stage of labour.	6
Q: 5:	State five (5) complications of occipitoposterior position to the baby.	5
Q: 6:	State five (5) characteristics of puerperium.	5
Q: 7:	State six (6) clinical manifestations of puerperal psychosis.	6
Q: 8:	State five (5) roles of the midwife in the first home visit during puerperium.	5

PART III: LONG ANSWER QUESTIONS. **MARKS**

Q: 1: Mrs. Z a primigravida at term, is admitted in the labour ward with a diagnosis of true labour. On vaginal examination, the cervix is 5cm dilated and a partograph is started to monitor the progress of labour.

a)	State five (5) uterine actions in the physiology of the first stage of labour.	5
b)	Describe the management of Mrs. Z until full cervical dilatation.	12
c)	State three (3) factors that influence the outcome of labour.	3

Q: 2: Mrs. X. aged 26 years is admitted in the postnatal with complaints of trauma during birth.

a)	List four (4) types of trauma during birth.	2
b)	Explain three (3) physiological observations in post natal care.	6
c)	Describe the specific management of a post natal mother who has had a traumatic birth.	10
d)	State two (2) complications of birth trauma to the baby.	2

Q:3: Mrs. K. a primigravida is admitted to the labour ward at 42 weeks gestation in the active phase of labour. During second stage she is diagnosed with shoulder dystocia.

a) State three (3) signs of shoulder dystocia. 3

b) Describe the management of Mrs. K. using three (3) non-invasive procedures until completion of the third stage of labour. 12

PART II: SHORT ANSWER QUESTIONS **MARKS**

Q1: State five (5) signs and symptoms of gonococcal infection in a pregnant woman. 5

Q2: State five (5) features identifiable during obstetric history taking that are likely to have an impact on future pregnancies. 5

Q3: State three (3) principles common to all mechanisms of labour. 3
Q4: State five (5) complications associated with deep transverse arrest. 5
Q5: State five (5) aims of immediate care of a mother with eclampsia. 5
Q6:State five (5) reflexes present in a normal term baby. 5

Q7: State three (3) causes of hypothermia in a newborn. 3

Q8: State five (5) roles of the midwife in promoting post natal psychological health. 5

Q9: Outline four (4) factors that have an impact on maternal nutrition that you would consider in infant feeding counseling for the HIV positive mothers. 4

PART III: LONG ANSWER QUESTIONS (ESSAY) **MARKS**

Q1: Mrs. Mark is admitted in the ward three weeks following delivery with a diagnosis of post partum depression.

a) Define post partum depression. 1

b) State five (5) predisposing factors to post partum depression. 5

c) Outline four (4) clinical manifestations of post partum depression. 4

d) Describe the management of Mrs. Mark till discharge. 10

Q2: Mrs. Peres, para 2+0 gravida 3 is admitted to the labour ward with complains suggestive of labour. A confirmatory vaginal examination is done which reveals brow presentation in active phase of labour with cervical dilatation of 4cm.

a) State three (3) causes of brow presentation. 3

b) Describe the management of Mrs. Peres. 14

c) List six (6) complications of brow presentation. 3

PART II: SHORT ANSWER QUESTIONS **MARKS**

Q1: State five (5) risks of preterm- prelabour rupture of membranes. 5

Q2: State five (5) signs and symptoms of gonococcal infection in a pregnant woman. 5

Q3: Explain the active management of the third stage of labour. 6

Q4: State five (5) contraindications of external cephalic version. 5

Q5: Draw a well labeled diagram of a lactating breast. 5

Q6: Outline the management of a baby undergoing phototherapy. 5

Q7: State four (4) differences between caput succedaneum and cephalohematoma. 4

Q8: State five (5) health messages you would share with a breastfeeding mother during a post natal visit. 5

PART III: LONG ANSWER QUESTIONS (ESSAY) **MARKS**

Q1: Mrs. Mark para 1+0 gravida 2 is admitted in labour ward with complains of abdominal pain at term. On vaginal examination the cervix is 6cm dilated. A partograph is

started to monitor the progress of labour.

a) State five (5) physiological changes that take place during the first stage of labour. 5

b) Describe the twelve (12) components of a partograph. 12

c) List six (6) possible complications of the first stage of labour. 3

Q2: Safe motherhood initiative is a responsibility of the entire society and it concerns every health worker. To prevent maternal mortality there is need of proper understanding of what it is all about.

a) Define maternal death. 1

b) Outline four (4) aims of community midwifery. 4

c) State five (5) causes of direct obstetric deaths. 5

d) Describe nursing interventions used to prevent maternal deaths. 10

SHORT ANSWER QUESTIONS. **MARKS**

Q: 1: Draw a well labeled diagram of the fetal circulation showing the temporary structures. 5

Q: 2: State five (5) presumptive or possible causes of pregnancy and their differential diagnosis. 5

Q: 3: Explain three (3) obstetric events that may trigger disseminated intravascular coagulation. 6

Q: 4: State three (3) causes of prolonged labour. 3

Q: 5: Outline five (5) measures employed to ensure that a new born baby has adequate rest after birth. 5

Q: 6: State four (4) differences between caput succedaneum and cephalohematoma. 4

Q: 7: State five (5) roles of the midwife in the first home visit during puerperium. 5

Q: 8: a) List four (4) causes of breast engorgement. 2

b) Briefly describe the normal physiology of lactation. 5

LONG ANSWER QUESTIONS. **MARKS**

Q: 1: During an outreach activity organized by the family health care unit, you are requested to give health education to the clients about pillars of safe motherhood.

a) Define safe motherhood. 1

b) Explain the pillars of safe motherhood. 16

c) State three (3) roles of the nurse in outreach clinical activities. 3

Q: 2: Mrs. X a primigravida is admitted in the maternity unit with a diagnosis of pre-eclampsia at 36 weeks gestation.

a) Define pre-eclampsia. 1

b) State five (5) predisposing factors to pre-eclampsia. 5

c) Describe the management of Mrs. X till discharge. 12

d) List four (4) possible complications that are likely to occur to Mrs. X. 2

Short answer questions

Q: 1: State five (5) warning signs suggestive of congenital heart disease in the newborn.

Q:2 : State three (3) indications for cord blood sampling

Q: 3: Outline three (3) factors that may trigger disseminated intravascular coagulation in pregnancy.

Q: 4: Outline three (3) effects of preeclampsia to the fetus:-

Q: 5: State three (3) diagnostic findings of multiple pregnancy on abdominal palpation

Q: 6: State five (5) differences between true and spurious labour

PART II: SHORT ANSWER QUESTIONS. **MARKS**

Q: 1: State four (4) key components of safe motherhood. 4

Q: 2: Draw a well labelled diagram of a fetal skull showing the diameters. 5

Q: 3: State five (5) definite indications for emergency caesarean section. 6

Q: 4: State five (5) differences between true and spurious labour. 5

Q: 5: Explain the three (3) stages involved in bilirubin conjugation. 6

Q: 6: State five (5) predisposing factors of post partum haemorrhage. 5

Q: 7: State four (4) observations made when examining a placenta. 4

Q: 8: State five (5) challenges of community midwifery. 5

PART III: LONG ANSWER QUESTIONS (ESSAY) **MARKS**

Q: 1: Mrs Y a gravida 2, para 1+0 is admitted to the maternity unit with complains of vaginal bleeding at 28 weeks. A diagnosis of placenta praevia is made.

a) Define antepartum hemorrhage. 1

b) State the degrees of placenta praevia. 4

c) Describe the management of Mrs Y from admission till discharge. 12

d) List six (6) complications of placenta praevia. 3

Q: 2: Baby K is admitted to the special baby care unit with a diagnosis of prematurity.

b) Define prematurity. 1

b) State five (5) causes of prematurity. 5

c) Describe the management of baby K from admission until discharge. 12

d) State two (2) preventive measures of prematurity. 2

PART II: SHORT ANSWER QUESTIONS. **MARKS**.

Q: 1: State three (3) indicators of fetal well being during pregnancy. 3

Q: 2: Outline three (3) factors that may trigger disseminated intravascular coagulation in pregnancy. 3

Q: 3: State five (5) causes of atonic uterus. 5

Q: 4: Outline four (4) causes of the internal rotation during labour. 4

Q: 5: State five (5) indications for an episiotomy. 5

Q: 6: State three (3) situations where expressing breast milk is appropriate. 3

Q: 7: State five (5) warning signs suggestive of congenital heart disease in the newborn. 5

Q: 8: State three (3) indications for cord blood sampling. 3

Q: 9: Draw a well labeled diagram of a fetal skull showing the diameters. 5

Q: 10: State four (4) health messages you would share with a breastfeeding mother during a post natal visit. 4

PART III: LONG ANSWER QUESTIONS (ESSAY) **MARKS**

Q: 1: Mrs. Y para 1+0, gravida 2 is admitted to the labour ward in latent phase of labour. Upon spontaneous rupture of membranes a diagnosis of cord prolapse is made.

a) State five (5) causes of cord prolapse. 5

b) Describe the management of Mrs. Y for the first 12 hours. 13

c) List four (4) complications of cord prolapse. 2

Q: 2: Immediately after birth, the nurse midwife uses the knowledge of the baby's transitional requirements and capabilities to facilitate appropriate preparations for reception into the world or extra-uterine life.

a) State two (2) adaptation mechanisms of the newborn to extra-uterine life. 2

b) Describe the immediate care of the baby at birth 12

c) Explain the principles of general care of the baby 6

SHORT ANSWER QUESTIONS. **MARKS**

Q: 1: State the first three (3) components of Leopold's maneuvers. 3

Q: 2: Draw a well labeled diagram of the fetal skull showing landmarks of obstetric importance. 5

Q: 3: State five (5) predisposing factors to cord prolapse. 5

Q: 4: Explain the components of active management of third stage of labour. 6

Q: 5: State five (5) complications of occipitoposterior position to the baby. 5

Q: 6: State five (5) characteristics of puerperium. 5

Q: 7: State six (6) clinical manifestations of puerperal psychosis. 5

Q: 8: State five (5) roles of the midwife in the first home visit during puerperium. 5

SHORT ANSWER QUESTIONS.

MARKS

Q: 1: Draw a well labeled diagram of the fetal circulation showing the temporary structures. 5

Q: 2: State five (5) presumptive or possible causes of pregnancy and their differential diagnosis. 5

Q: 3: Explain three (3) obstetric events that may trigger disseminated intravascular coagulation. 6

Q: 4: State three (3) causes of prolonged labour. 3

Q: 5: Outline five (5) measures employed to ensure that a new born baby has adequate rest after birth. 5

Q: 6: State four (4) differences between caput succedaneum and cephalohematoma. 4

Q: 7: State five (5) roles of the midwife in the first home visit during puerperium. 5

Q: 8: a) List four (4) causes of breast engorgement. 2

b) Briefly describe the normal physiology of lactation. 5

PART II: SHORT ANSWER QUESTIONS. **MARKS**.

Q: 1: State three (3) indicators of fetal well being during pregnancy. 3

Q: 2: Outline three (3) factors that may trigger disseminated intravascular coagulation in pregnancy. 3

Q: 3: State five (5) causes of atonic uterus. 5

Q: 4: Outline four (4) causes of the internal rotation during labour. 4

Q: 5: State five (5) indications for an episiotomy. 5

Q: 6: State three (3) situations where expressing breast milk is appropriate. 3

Q: 7: State five (5) warning signs suggestive of congenital heart disease in the newborn. 5

Q: 8: State three (3) indications for cord blood sampling. 3

Q: 9: Draw a well labeled diagram of a fetal skull showing the diameters. 5

Q: 10: State four (4) health messages you would share with a breastfeeding mother during a post natal visit. 4

PART II: SHORT ANSWER QUESTIONS **MARKS**

1. Draw and label a diagram of the superficial muscle layers of the pelvic floor. 5
2. State five (5) diagnostic features of Occipital posterior position(OPP) on abdominal examination. 5
3. State five (5) key elements in determining the ripening of the cervix. 5
4. Outline the uterine action in the physiology of first stage of labour. 5
5. State six (6) risks of preterm prelabour rupture of membranes. 6
6. State four (4) functions of the placenta. 4
7. State how Apgar score is used in the diagnosis of asphyxia neonatorum. 5

8. State five (5) roles of a domiciliary midwife. 5

PART III: LONG ANSWER QUESTIONS (ESSAY) **MARKS**

1. The incidence of multiple pregnancy is currently on the raise due to various kinds of treatments for infertility.
   a) State the types of multiple pregnancy. 2
   b) State four differences between the types of multiple pregnancies stated above. 4
   c) Describe the management of normal labour in multiple pregnancy. 14
2. Anaemia is one of the medical conditions that complicate pregnancy.
   a) State five (5) predisposing factors to anaemia in pregnancy. 5
   b) Describe the management of moderate anaemia during pregnancy. 12
   c) Outline three (3) effects of anaemia on pregnancy. 3

PART II: SHORT ANSWER QUESTIONS **MARKS**

1. Explain three (3) possible complications associated with multiple pregnancy. 6
2. State six (6) health messages a midwife would share with a HIV positive mother who is breastfeeding. 6
3. Define the following terms:-
   a) Moulding. 1
   b) Effacement. 1
   c) Anencephaly. 1
   d) Meningocele. 1
4. Differentiate between the latent and active phases of labour. 4
5. Outline three (3) factors that govern the choice of position and mobility during labour. 3
6. State five (5) past histories that are relevant at the onset of labour. 5
7. Puerperium is a very critical period after a woman gives birth to the infant.
   a) Outline three (3) roles of a midwife during puerperium. 3
   b) State four (4) complications associated with puerperium. 4
8. Draw a well labelled diagram to illustrate the internal female genitalia. 5

PART III: LONG ANSWER QUESTIONS (ESSAY) **MARKS**

1. Obstructed labour is one of the causes of direct maternal deaths.
   a) State five (5) causes of obstructed labour. 5
   b) Describe the management of obstructed labour. 12
   c) State three (3) maternal complications of obstructed labour. 3

2. Respiratory distress syndrome (RDS) is the commonest cause of neonatal respiratory disease and a common cause of neonatal and infant mortality.
   a) Define respiratory distress syndrome. 1
   b) State four (4) predisposing factors to respiratory distress syndrome. 4
   c) Outline three (3) cardinal signs of respiratory distress syndrome. 3
   d) Describe the management of respiratory distress syndrome.

3. Acute inversion of the uterus is a rare but a potentially life threatening complication of third stage of labour.
   a) State the classification of uterine inversion according to severity. 3
   b) Outline five (5) causes of uterine inversion. 5
   c) State two (2) manifestations of acute uterine inversion. 2
   d) Describe the management of acute uterine inversion. 10

4. Post partum haemmorrhage is the most common maternal morbidity and a major cause of death worldwide.
   a) Define primary postpartum haemmorrhage. 1
   b) Outline five (5) predisposing factors to PPH. 5
   c) Describe the management of PPH. 12
   d) State two (2) ways of preventing PPH. 2

PART II: SHORT ANSWER QUESTIONS **MARKS**

Q: 1: List six (6) risk factors for premature rupture of membranes. 3

Q: 2: State five (5) temporal structures in fetal circulation. 5

Q: 3: Explain the cardiovascular physiological changes during pregnancy. 8

Q: 4: Outline five (5) anatomical variations of the placenta. 5

Q: 5: Explain the benefits of prevention of mother to child transmission to the:-

a) Mother 3
b) Baby 3
c) Community 3

Q: 6: Sequentially outline movements in normal labour. 7

Q: 7: State five (5) Kenyan laws governing domiciliary practice. 5

Q: 8: State four (4) objectives of antenatal visits. 4

Q: 9: Draw a well labeled diagram showing regions and landmarks of obstetrical importance in the fetal skull. 5

PART III: LONG ANSWER QUESTIONS (ESSAY) **MARKS**

Q: 1: Baby Lee is brought in the neonatal intensive care unit with a diagnosis of Asphyxia neonatorum.

a) Define the term asphyxia. 1

b) List six (6) predisposing factors to this condition. 3

c) Outline (4) clinical symptoms that baby Lee may present with. 4

d) Describe the management of baby Lee from admission till discharge. 12

Q: 2: Krista para 0+0 is at 42 weeks of gestation. She has been admitted into labour ward for induction of labour.

a) State four (4) maternal indications of induction of labour. 4

b) Describe pre and post nursing care following induction of labour with oxytocin. 8

c) Other than the method mentioned here, describe other ways of inducing labour. 8

SAQ'S

1. Define the following terms in labour. 6
   a) Position
   b) Presentation
   c) Lie
   d) Attitude
   e) Denominator
   f) Presenting part
2. Describe the nursing management of fetal distress. 6
3. State four (4) possible complications of breech presentation. 4
4. Draw a well labeled diagram of the superficial muscles of the pelvic floor. 5
5. Explain the health education you would give to a client with nausea and vomiting in pregnancy. 6
6. Describe three (3) sequential changes that lochia undergoes during puerperium. 6
7. Draw a well labeled diagram showing regions and landmarks of obstetrical importance in the fetal skull. 5

LAQ

1. Mrs.Kijana is admitted in labour ward in latent phase of labour. Discuss the use of partogram in monitoring labour. 20